

IN THE UNITED STATES DISTRICT COURT
FOR THE NORTHERN DISTRICT OF TEXAS — FORT WORTH DIVISION
Copeland v. Weinstein Management Co., Inc., et al.

EXHIBIT

2

**Dr. Isabel S. Huang Letter of Support, September 10,
2025 (UT Southwestern PM&R)**

Legal Element Supported:

Initial medical baseline; treating provider since October 8, 2024

Verified Complaint Reference:

¶ 18

UTSouthwestern Medical Center

University Hospital Physical Medicine & Rehabilitation Clinic
Clinic Phone: 214-645-2080

September 8, 2025

Patient Name: Kathryn Copeland
DOB: June 28, 1985

RE: Medical Statement for Disability Application

To Whom It May Concern,

I am writing this statement in support of Kathryn Copeland's application for Social Security Disability Insurance and Supplemental Security Income. She has been under my care at UTSouthwestern since October 8, 2024, for Hypermobility EhlersDanlos Syndrome (hEDS), Mast Cell Activation Syndrome (MCAS) and Postural Orthostatic Tachycardia Syndrome (POTS). These are severe chronic illnesses that significantly impair her daily functioning and ability to sustain employment.

In addition to these primary conditions, review of her medical records shows she is also being treated for the following chronic and compounding illnesses: Long COVID, Median Arcuate Ligament Syndrome (MALS), a vestibular schwannoma (brain tumor), Hashimoto's disease with severely low iron requiring three iron infusions, bilateral vision loss and binocular dysfunction linked to toxic mold exposure, depression, post-traumatic stress disorder (PTSD), and immune abnormalities including hypogammaglobulinemia.

Her July 2025 evaluation by Dr. Alfred Johnson documented toxic mold exposure with extremely elevated levels of Aspergillus, Chaetomium, and Stachybotrys. He noted severe chronic fatigue (up to 18 hours/day bedridden), 30 lbs of weight loss, cognitive impairment, and hyperbaric oxygen therapy and urgent need for relocation.

Since my prior March 2025 statement, Ms. Copeland's health has further deteriorated. She was displaced in June/July 2025 due to mold contamination and has been forced to live in a hotel. This unstable housing situation prevents her from accessing home health nursing and supportive services previously prescribed, further worsening her decline.

She continues to experience severe and disabling symptoms, including:

- Severe systemic inflammation.

- Persistent fatigue and post-exertional malaise.
- Orthostatic intolerance, dizziness, tremors, and frequent migraines.
- Neurological and sensory issues, including visual disturbances and light sensitivity.
- Severe GI impairment with ongoing nutritional instability.
- Cognitive dysfunction, including impaired memory, slowed processing, and executive dysfunction.

KATHRYN MARIE COPELAND (MR# 97531555)
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Her PROMIS-29 scores from September 2025 confirm extremely poor physical function, high fatigue, high pain interference, and significant mood symptoms. Despite treatment efforts, her condition remains chronic, disabling, and unlikely to improve.

Without access to disability benefits, Ms. Copeland is unable to secure safe housing, purchase adequate food, obtain necessary medications, or pay for transportation to medical appointments. This lack of resources is directly worsening her health and preventing her from accessing prescribed care.

In my professional opinion, Ms. Copeland remains unable to perform sustained work activity and is in urgent need of financial and medical support to stabilize her living conditions and prevent irreversible decline.

Sincerely,

 9/10/25

Isabel Huang, MD
UT Southwestern Medical Center

Patient Name: Kathryn Copeland

DOB: June 28, 1985

Date: September 11, 2025

Provider Name & Credentials: Kim Tran, M.D.

Facility: UT Southwestern Medical Center
600 S Main St 3rd Floor, Suite 500
Fort Worth, TX 76104

Dire Needs Letter

To Whom It May Concern,

I am the primary care physician for Kathryn Copeland. She has been under my care for multiple chronic medical conditions that significantly impair her daily functioning, including hypermobile Ehlers-Danlos Syndrome (hEDS), Postural Orthostatic Tachycardia Syndrome (POTS), Mast Cell Activation Syndrome (MCAS), Long COVID/ME-CFS, Median Arcuate Ligament Syndrome (MALS), Hashimoto's thyroid disease, immune system abnormalities (hypergammaglobulinemia), a vestibular schwannoma, and complications from toxic mold exposure.

These conditions collectively cause severe fatigue, post-exertional malaise, autonomic instability, chronic pain, tremors, migraines, visual disturbances, and cognitive dysfunction (slowed processing, memory impairment, and brain fog). Even minimal exertion can trigger symptom flares requiring extended rest.

Since early 2025, Ms. Copeland's health has worsened. She was displaced from her home in June/July 2025 due to toxic mold and has been forced to live in unstable hotel housing. This disruption prevents her from accessing prescribed home health services, therapy, and ongoing medical care, further compromising her health.

In my professional medical opinion, Ms. Copeland is unable to sustain an eight-hour workday, five days per week, on a regular and continuing basis. She requires frequent unscheduled breaks, must recline or lie down throughout the day, and would miss multiple days of work each month due to her conditions. These limitations are consistent with the updated Residual Functional Capacity Questionnaire I completed on September 10, 2025.

Without disability benefits, Ms. Copeland cannot maintain safe housing, afford adequate food, secure medications, or access consistent treatment. This lack of resources is directly worsening her condition and places her at risk of irreversible decline.

It is my medical opinion that Ms. Copeland remains unable to perform substantial gainful activity and is in urgent need of disability benefits to stabilize her health and living situation.

Sincerely,



Kim Tran, M.D
UT Southwestern Medical Center

Residual Functional Capacity Questionnaire PHYSICAL RESIDUAL FUNCTION CAPACITY

Patient: Kathryn CopelandDOB: 6/28/1985Physician completing this form: Dr. Kim Tran

Please complete the following questions regarding this patient's impairments and attach all supporting treatment notes, radiologist reports, laboratory and test results.

Symptoms & Diagnosis

What diagnoses has this patient received? Hypergammaglobulinemia (immune dysfunction),Long COVID, Fibromyalgia, Hypermobility EDS, POTS, MCAS, MALS, Hashimoto's, Toxic mold exposure with visual impairment & cognitive decline, vestibular schwannoma, depression, PTSDDescribe the patient's symptoms, such as pain, dizziness, fatigue, etc. Post-exertional malaise,chronic pain (muscle, joint, neuropathic), orthostatic intolerance, memory issues, slowed processing, chronic headaches, unrefreshing sleep, sensory sensitivities, visual decline, fatigue (bed 18 hrs/day)Does the patient have chronic pain/paresthesia? ☒ Yes ☐ No

Describe the patient's type of pain, location, frequency, precipitating factors, and severity. _____

Chronic widespread pain affecting muscles, joints, and nerves. Frequent neuropathic pain (burning, tingling) and joint instability pain (due to hEDS). Pain is daily and worsens with activity, standing, overexertion, or prolonged sitting. Severity varies but is often debilitating, requiring rest and medication for management.

Please indicate all positive objective signs exhibited by the patient:

☐ Decreased range of motion (list specific joints): _____☐ Crepitus☐ Joint Swelling☒ Spasms☐ Abnormal gait☒ Impaired appetite☒ Reduced grip strength☐ Joint Deformity☐ Joint Redness☒ Weakness☒ Abnormal posture☒ Impaired sleep☒ Joint Instability☐ Joint Warmth☐ Trigger points☒ Fatigue☒ Malaise☒ Sensory changes☒ Joint Tenderness☒ Atrophy☐ Reflex changes☐ Fever☐ Positive straight leg test☒ Weight loss (Involuntary)What is the earliest date that the above description of limitations applies? December2022Have these symptoms lasted (or are they expected to last) twelve months or longer? ☒ Yes ☐ NoAre this patient's symptoms and functional limitations impacted by emotional factors? ☒ Yes ☐ No

If yes, please mark any known psychological conditions that affect this patient's pain:

☒ Depression☒ Anxiety☐ Somatoform disorder☐ Personality disorder☒ Other: ADHD,PTSDAre these physical and emotional impairments reasonably consistent with the patient's symptoms and functional limitations? ☒ Yes ☐ No

If no, please explain: Katie's physical and emotional impairments are fully consistent with her symptoms and functional limitations. Her severe fatigue, post-exertional malaise (PEM), cognitive dysfunction, chronic pain, and orthostatic intolerance (POTS) are well-documented and align with her inability to sustain work, perform daily tasks, or engage in routine activities without severe consequences.

Testing & Treatments

Identify any positive clinical findings and test results: _____

Compass 31 Score: 58.7 (indicating significant autonomic dysfunction/POTS), abnormal neurocognitive testing (documented memory impairment, slowed processing speed), abnormal vestibular testing (balance deficits), MRI confirming vestibular schwannoma, and unrefreshing sleep consistent with disrupted sleep architecture.

Please list the patient's current medications: Levothyroxine 50 mcg, Adderall XR (30 mg), Namenda 10 mg twice daily, Sertraline 100 mg, Gabapentin 300 mg three times daily, Auvelity (Dextromethorphan-Bupropion) 45-105 mg twice daily, Low-Dose Naltrexone 4.5 mg, Zyrtec 10 mg, Singulair 10 mg twice, Trazodone 100 mg nightly, Prazosin 3 mg nightly, Clonazepam 0.5 mg as needed, Propranolol 20 mg as needed, Potassium

Please indicate the treatment type, start dates, and frequency: Katie's treatment includes medication management (ongoing since diagnosis), physical therapy, cognitive therapy, dietary modifications for MALS and Celiac Disease, and autonomic dysfunction management (fluids, salt intake, compression garments). She started Long COVID treatment in June 2023, began cognitive therapy in late 2023, and continues medication therapy daily. She follows up with specialists, including neurology, rheumatology, psychiatric, and physical rehabilitation medicine, every 3-6 months.

What is the patient's prognosis? Katie's condition is chronic and disabling, with no expected significant improvement despite treatment.

Is this patient a malingering? ☐ Yes ☒ No

Functional Work Limitations

When answering the following questions, please consider this patient's impairments and estimate his or her ability to work in a competitive work environment for an 8-hour shift with normal breaks.

How often do you expect this patient's pain or symptoms to interfere with the attention and concentration necessary to perform simple work tasks?

- ☐ Never
☐ Rarely (1% to 5% of an 8 hour working day)
☐ Occasionally (6% to 33% of an 8 hour working day)
☐ Frequently (34% to 66% of an 8 hour working day)
☒ Constantly

How well do you expect this patient to be able to tolerate work stress?

- ☒ Incapable of even "low stress" jobs
☐ Only capable of low stress jobs
☐ Moderate stress is okay
☐ Capable of high stress situations

Explain: Stress worsens POTS, fatigue, pain, and cognitive dysfunction, triggering crashes that last for days. She cannot sustain focus or physical endurance, even with accommodations.

Is this patient taking any medications with side effects that may affect his or her ability to work?

☒ Yes ☐ No

If yes, please list possible side effects. Katie's medications cause significant side effects, including cognitive impairment, brain fog, drowsiness, dizziness, nausea, and fluctuations in blood pressure. These side effects further limit her ability to concentrate, complete tasks, and maintain a consistent

How far can this patient walk without rest or severe pain? Less than 200 feet

How long can this patient sit comfortably at one time before needing to get up?

Minutes: 0 5 10 15 20 30 45

Hours: 1 2 Longer than 2

What must the patient usually do after sitting this long?

☐ Stand ☐ Walk ☒ Lie Down ☒ Other:

She can sit comfortably for 10-15 minutes before needing to change positions or lie down due to pain, circulatory issues, and muscle weakness.

How long can this patient stand comfortably at one time before needing to sit or walk around?

Minutes: 0 ☒ 5 10 15 20 30 45

Hours: 1 2 Longer than 2

What must the patient usually do after sitting this long? After sitting for this duration, she must lie down or
☒ Sit ☐ Walk ☒ Lie Down ☐ Other: recline to alleviate symptoms and prevent worsening dysautonomia and post-exertional malaise.

How long can this patient sit in an 8-hour working day?

- ☒ less than 2 hours
☐ about 2 hours
☐ about 4 hours
☐ at least 6 hours

How long can this patient stand and/or walk in an 8-hour working day?

- ☒ less than 2 hours
☐ about 2 hours
☐ about 4 hours
☐ at least 6 hours

Does this patient need to include periods of walking in an 8-hour working day?

☒ Yes ☐ No

If yes, how long? 5 10 15 20 ☒ 30 45 60 90 minutes

How often? 1 2 3 4 5 6 7 8 9 ☒ 10

Does this patient require a job that allows the opportunity to change between sitting, standing and walking at will? ☒ Yes ☐ No

Does this patient require unscheduled breaks?

☒ Yes ☐ No

If yes, how often? She needs the ability to change positions at will and take more than 10 breaks per day, lasting 20-30 minutes each.

During this time, this patient will need to ☒ lie down ☐ sit quietly for 20-30 minutes.

With prolonged sitting, should this patient's leg(s) be elevated?

☒ Yes ☐ No

If yes, for what percentage of time in an 8-hour day? 50 %

During occasional standing/walking, does this patient require a cane or other assistive device?

☒ Yes ☐ No

How many pounds can this patient lift and carry? **Fatigue and joint instability make less unpredictable.**

	Never	Rarely	Occasionally	Frequently
Less than 10 lbs.	☐	☐	☒	☐
10 lbs.	☐	☒	☐	☐
20 lbs.	☒	☐	☐	☐
50 lbs.	☒	☐	☐	☐

How often can your patient perform the following activities?

	Never	Rarely	Occasionally	Frequently
Twist	☐	☒	☐	☐
Stoop (bend)	☐	☒	☐	☐
Crouch	☐	☒	☐	☐
Climb ladders	☒	☐	☐	☐
Climb stairs	☐	☒	☐	☐

Does this patient have significant limitations with repetitive reaching, handling or fingering?

☒ Yes ☐ No

If yes, please indicate the percentage of time this patient can perform the following activities:

Using hands to grasp, turn and twist objects	Right	<u>10</u>	%	Left	<u>10</u>	%
Using fingers for fine manipulation	Right	<u>10</u>	%	Left	<u>10</u>	%
Using arms to reach out and overhead	Right	<u>5</u>	%	Left	<u>5</u>	%

Note: Tremors, vision issues, and joint instability further limit fine motor tasks and overhead reaching.

Are this patient's impairments likely to produce "good days" and "bad days"?

☒ Yes ☐ No

If yes, please estimate, on average, how many days per month your patient is likely to be absent from work as a result of the impairments or treatment:

☐ Never	☐ About three days per month
☐ About one day per month	☐ About four days per month
☐ About two days per month	☒ More than four days per month

Please describe any other limitations that might affect this patient's ability to work at a regular job on a sustained basis, such as psychological issues, limited vision or hearing, or the inability to adjust to temperature, wetness, humidity, noise, dust, fumes, gases or hazards, etc.

Katie's impairments also cause: Cognitive dysfunction, brain fog, and memory issues, difficult to retain new information; Sensitivity to light, sound, mold, and temperature, limiting her ability to work in typical office environments; Autonomic dysfunction, causing unpredictable dizziness, and sudden drops in blood pressure; and Inability to regulate body temperature, meaning exposure to heat, humidity, or cold can trigger symptom flares.

Severe brain fog, slowed processing, memory loss, and executive dysfunction confirmed by neuropsychological testing and PROMIS-29 scores (Sept 2025).

Please describe additional tests or clinical findings not described on this form that clarify the severity of the patient's impairments.

Executive dysfunction, brain fog, memory impairment, and slowed processing speed consistent with Long COVID and ME/CFS-related cognitive impairment. Orthostatic Vital Signs: Show increased heart rate upon standing, consistent with dysautonomia and blood pooling. Cardiovascular Testing (if applicable): Shows dysautonomia-related circulation issues, contributing to fatigue and exercise intolerance. Immune & Inflammatory Markers: Evidence of immune dysfunction (MCAS, chronic inflammation), contributing to fatigue, and allergic-like reactions. Gastrointestinal Studies (MALS and MCAS): Shows delayed gastric emptying, vascular compression, or food-triggered reactions, leading to severe fatigue,

Chronic, worsening despite treatment; prognosis poor without stable housing and consistent medical care.

Completed by:

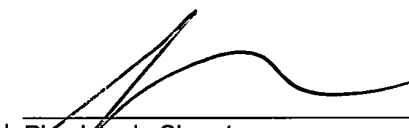
Kim Tran, M.D.

Physician's Printed Name

600 S. Main St., Suite 500

Address
3rd Floor

Fort Worth, Texas 76104


Physician's Signature

9-11-25
Date